NCCP 00431 · VERSION 11

Your treatment, explained

NCCP 00431a: advanced unresectable or metastatic melanoma in adults. This patient guide is the melanoma context, not the separate liver-cancer indication. · NCCP 00431 v11 · Document revised 6 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Nivolumab and ipilimumab for advanced melanoma

Reference treatment cycle

Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your treatment and urgent-contact card is supplied separately and in the wallet-card section of the complete handbook.

Sources: nccp

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

How each drug works
The overall treatment plan
Your 21-day timeline
Baseline and cycle-specific checks
Whole treatment, scans and checks
Tablets, visits and practical planning
Understanding your diagnosis and treatment goal
Your medicines and the overall plan
Taking treatment and preparing for visits
Tests: what is required and what each result means
What to expect between visits
Skin checks, sun protection and optional photographs
Pregnancy, fertility and protecting others
Questions, decisions and practical support
Follow-up and your continuing record
Supportive medicines
Understanding the evidence
Questions to ask
Sources

UNDERSTAND YOUR TREATMENT

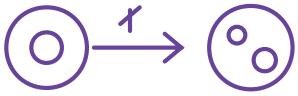
Medicines in this regimen

Nivolumab and ipilimumab help immune cells recognise and attack melanoma. The combination phase is followed by nivolumab alone when appropriate. The aim in this setting is to control advanced disease; benefit and toxicity are reviewed for the individual.

PD-1 CHECKPOINT IMMUNOTHERAPY

Nivolumab

Opdivo



Releases a PD-1 immune brake

Nivolumab blocks PD-1, a signal that can reduce T-cell activity. This can help immune cells act against melanoma, but it can also allow inflammation in healthy organs. It is not conventional chemotherapy and side effects can occur even after doses have stopped.

PD-1 blocked → Immune activity increases → Healthy organs can inflame

What to watch for

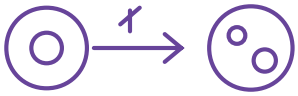
Carry your immunotherapy alert card and report new symptoms early, including after treatment ends.

Sources: nccp · nccn-patient

CTLA-4 CHECKPOINT IMMUNOTHERAPY

Ipilimumab

Yervoy



Releases a second immune brake

Ipilimumab blocks CTLA-4, another checkpoint involved in controlling T cells. Combining it with nivolumab increases immune activity and can increase immune-related toxicity. The four-dose combination phase has a different treatment plan from nivolumab maintenance.

CTLA-4 blocked → Combined immune activation → Four-dose induction phase

What to watch for

New diarrhoea, abdominal pain, severe headache or unusual weakness needs early contact. Do not wait for the next infusion.

Sources: nccp · nccn-patient

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The anatomical side-effect map follows.

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

1 Hormones / head / eyes

Nivolumab / ipilimumab

New severe headache, visual change, unusual tiredness, dizziness, thirst or frequent urination may signal immune effects and needs prompt advice.

3 Heart / muscles

Nivolumab / ipilimumab

Chest pain, palpitations, marked muscle weakness or swallowing difficulty can be serious immune effects. Seek urgent assessment.

5 Kidneys / urine

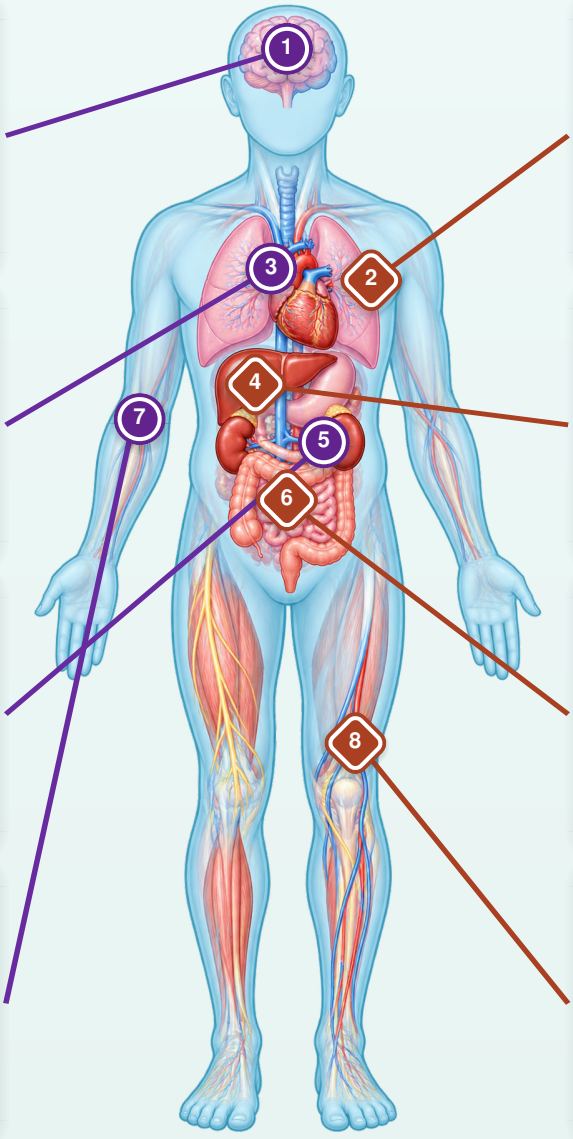
Nivolumab / ipilimumab

Kidney inflammation may be detected on blood tests. Report reduced urine, swelling, blood in urine or a marked change from normal.

7 Skin / mouth

Nivolumab / ipilimumab

Report rash or itching early. Blistering, peeling, mouth or genital sores, or rash with fever needs urgent assessment.



2 Lungs / breathing

Nivolumab / ipilimumab

New cough or breathlessness may reflect lung inflammation or another serious cause. Call promptly; severe breathing difficulty is an emergency.

4 Liver

Nivolumab / ipilimumab

Inflammation may first show in blood tests. Report yellow eyes or skin, dark urine, unusual itching or upper-abdominal pain.

6 Bowels / abdomen

Nivolumab / ipilimumab

Report increased stool frequency, diarrhoea, blood or mucus, or abdominal pain early. Severe pain, black stools or dehydration needs urgent help.

8 Nerves / strength

Nivolumab / ipilimumab

New tingling, weakness, unsteadiness, facial change or trouble swallowing needs prompt review. Sudden weakness or speech difficulty is an emergency.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

The overall treatment plan

UP TO FOUR VISITS

Combination induction

Nivolumab then ipilimumab | **Every 21 days, up to four doses**

Nivolumab 1 mg/kg and ipilimumab 3 mg/kg are given by IV infusion, subject to review before every dose. Four is the planned combination limit, not the total number of all treatment visits.

MAINTENANCE

Nivolumab alone

Every 2 or 4 weeks as prescribed | **IV or subcutaneous route as selected**

After the last melanoma combination dose, the first two-weekly nivolumab dose starts three weeks later or the first four-weekly dose six weeks later. The team confirms dose, route, interval and start date. Treatment continues while beneficial and tolerable; previous immune-toxicity decisions still apply.

The treatment setting determines duration; a reference calendar is not a prescription or a reason to restart a held medicine.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Required safety checks and your individual disease-assessment plan

The team supplies actual dates and resolves any source ambiguity. A reference period does not replace a prescription, a local monitoring plan or an individual treatment decision.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00431 v11: current regimen source · NCCN Guidelines for Patients: Melanoma, 2026 - supplied education reference

THE WHOLE TREATMENT JOURNEY

Nivolumab and ipilimumab for advanced melanoma: treatment and checks

NCCP 00431a: advanced unresectable or metastatic melanoma in adults. This patient guide is the melanoma context, not the separate liver-cancer indication.

The combination phase gives nivolumab followed by ipilimumab by IV infusion every three weeks for up to four doses. If appropriate, nivolumab alone then continues on the team-selected two-weekly or four-weekly schedule, by the prescribed IV or subcutaneous route. After the last combination dose, the first two-weekly maintenance dose starts three weeks later; the first four-weekly maintenance dose starts six weeks later in this melanoma schedule. Treatment continues until progression or unacceptable toxicity, subject to review.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Bloods and organ safety NCCP 00431	Full blood count, kidney and liver tests, glucose and thyroid tests; BRAF tumour testing and review of autoimmune conditions, transplantation, prior immune reactions and current medicines.	Full blood count, kidney/liver tests, glucose and thyroid tests each treatment cycle. Cortisol and other tests are added when clinically indicated. Every visit also reviews bowel, breathing, skin, endocrine, neurological and other symptoms; a normal blood test does not exclude immune toxicity.	Contact the team for new symptoms; extra tests may be needed between scheduled checks.	Safety bloods do not by themselves show whether cancer is responding.	Continue to report possible immune effects for at least five months after the last dose, as specified by NCCP, and for longer whenever your team advises. Keep any steroid taper, hormone-replacement plan and relevant follow-up tests. Response imaging follows your individual plan.
Cancer, skin and daily function NCCP 00431	Record diagnosis, stage, current disease sites and the baseline assessment.	Keep the source-specific skin and clinical reviews and the individually agreed response assessments.	Record new lesions, lumps, wound changes, pain and effects on usual activities.	Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed.	Follow-up depends on stage, treatment and symptoms. Your team records the skin/lymph-node examination plan, whether body or brain imaging is needed, and which service owns each appointment. A general patient booklet does not set a universal scan interval for every patient.
Medicines and continuing precautions NCCP 00431	Review all medicines and the pregnancy/fertility plan.	Review treatment taken or received, holds, adverse effects and supportive medicines.	Keep the actual urgent contacts and do not restart a held medicine yourself.	Bring the medicine list and treatment summary to other services.	Discuss fertility and pregnancy precautions before starting. Immunotherapy may harm a pregnancy. Use the contraception plan and after-treatment duration specified for the medicines you receive; ask about breastfeeding restrictions. Tell the team promptly about possible pregnancy. Do not stop necessary hormone replacement or prescribed steroids without advice.

What each check is for

1

Combination induction

Up to four visits

Nivolumab and ipilimumab help immune cells recognise and attack melanoma. The combination phase is followed by nivolumab alone when appropriate. The aim in this setting is to control advanced disease; benefit and toxicity are reviewed for the individual.

Ask / act: Confirm the current treatment phase, prescription, tests and support medicines.

Sources: NCCP 00431

2

Nivolumab alone

Maintenance

Nivolumab and ipilimumab help immune cells recognise and attack melanoma. The combination phase is followed by nivolumab alone when appropriate. The aim in this setting is to control advanced disease; benefit and toxicity are reviewed for the individual.

Ask / act: Confirm the current treatment phase, prescription, tests and support medicines.

Sources: NCCP 00431

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.40

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

The combination phase gives nivolumab followed by ipilimumab by IV infusion every three weeks for up to four doses. If appropriate, nivolumab alone then continues on the team-

selected two-weekly or four-weekly schedule, by the prescribed IV or subcutaneous route. After the last combination dose, the first two-weekly maintenance dose starts three weeks later; the first four-weekly maintenance dose starts six weeks later in this melanoma schedule. Treatment continues until progression or unacceptable toxicity, subject to review.

Treatment timing: displayed days 1–21

Treatment or check	Days in this window	What happens
Combination infusion	Day 1	Treatment after clinical review and the required tests.
Symptoms between visits	Days 1–21	Immune effects can begin at any time, including after treatment stops.
Next-cycle assessment	Day 21	The team confirms the next dose; maintenance has its own schedule.

Timing note 1: The combination phase gives nivolumab followed by ipilimumab by IV infusion every three weeks for up to four doses. If appropriate, nivolumab alone then continues on the team-selected two-weekly or four-weekly schedule, by the prescribed IV or subcutaneous route. After the last combination dose, the first two-weekly maintenance dose starts three weeks later; the first four-weekly maintenance dose starts six weeks later in this melanoma schedule. Treatment continues until progression or unacceptable toxicity, subject to review.

Timing note 2: If an appointment is missed or treatment is held, contact the unit for a new plan. Do not count a missed visit as a dose received. A move to maintenance does not erase previous immune toxicity or a permanent-stop decision.

Timing note 3: This is nivolumab1/ipilimumab3, not the lower-ipilimumab combination.

Timing note 4: Four induction doses do not mean the whole treatment course is four doses.

Timing note 5: Prior immune toxicity and permanent-stop decisions remain relevant when maintenance starts.

Timing note 6: No routine percentage dose reductions: the team may hold or stop immunotherapy and treat the adverse effect.

Why these bands stay broad: This calendar shows the pattern. Your team supplies actual dates, prescribed doses and any hold or restart instructions. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00431 v11: current regimen source ↗

There are no guaranteed "safe days". The prescribed dose, treatment decision and local urgent-contact plan take priority. Call 112 or 999 for severe breathing difficulty, severe chest pain, collapse, a new seizure, sudden weakness or speech difficulty, or uncontrolled bleeding. Follow your oncology unit's written urgent-contact plan for other new or worsening symptoms; do not wait for the next appointment.

Complete timing and symptom notes

Day 1: Treatment visit

At each combination visit, nivolumab is given before ipilimumab, as separate IV infusions. The source schedule gives each over 30 minutes; allow extra time for assessment, preparation and observation. Maintenance may be IV or subcutaneous nivolumab with its own dose and appointment interval. The route and interval are recorded in your plan; they are not interchangeable on your own. Report symptoms and previous reactions before treatment.

Day 2–21: Between visits

Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy. Do not wait for the next infusion to report a possible immune effect.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Taking or receiving treatment

At each combination visit, nivolumab is given before ipilimumab, as separate IV infusions. The source schedule gives each over 30 minutes; allow extra time for assessment, preparation and observation. Maintenance may be IV or subcutaneous nivolumab with its own dose and appointment interval. The route and interval are recorded in your plan; they are not interchangeable on your own.



STEP 2

Missed, vomited or held treatment

If an appointment is missed or treatment is held, contact the unit for a new plan. Do not count a missed visit as a dose received. A move to maintenance does not erase previous immune toxicity or a permanent-stop decision.

STEP 3

Appointments and supply

Arrange dates, medicine supply, transport and any needed interpreter or carer support. Ask whether bloods are taken in the oncology unit or elsewhere and how the result reaches the prescriber.

STEP 4

Paper copies and contact information

Keep this complete handbook, the three-page summary and the wallet card together. The QR code is optional. Ask the team to add your actual contacts and print a revised plan when treatment changes.

Sources: NCCP 00431 v11: current regimen source. Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 An individual symptom plan

Ask which anti-sickness or other symptom medicines are prescribed, when to take them and when to call if they do not help. Do not start a borrowed medicine or alter treatment to manage a side effect yourself.

2 Your urgent contacts

Keep your actual oncology, out-of-hours and pharmacy contacts on the wallet card. Example numbers on a template must be replaced before you use it.

3 Pregnancy and fertility precautions

Discuss fertility and pregnancy precautions before starting. Immunotherapy may harm a pregnancy. Use the contraception plan and after-treatment duration specified for the medicines you receive; ask about breastfeeding restrictions. Tell the team promptly about possible pregnancy. Do not stop necessary hormone replacement or prescribed steroids without advice.

4 Eating, activity and practical help

Report falling intake, weight loss, pain or fatigue early. The team can tailor nutrition, activity, wound, psychological and practical support to your needs.

5 Steroids and hormone replacement

Some immune effects need steroids or lasting hormone replacement. Follow the exact prescription and taper. Carry the relevant alert information, and do not stop or reduce these medicines yourself when anticancer treatment is held or completed.

Sources: nccp · nccn-patient

TREATMENT REFERENCE · 01

Understanding your diagnosis and treatment goal

Use these questions with your own pathology report and agreed treatment plan.

What the diagnosis says

Keep the pathology summary, original skin site, stage and details of any lymph-node involvement together. Breslow thickness and ulceration describe features of the original melanoma; they are different from the stage. If the diagnosis arose in the eye or a mucosal surface, ask whether this cutaneous-melanoma information applies.

Sources: nccn-patient

BRAF and other tumour tests

A BRAF V600 result helps decide whether the BRAF/MEK targeted medicines as a separate treatment option from the immunotherapy described in this guide are suitable. This is usually a test of the tumour, not proof of an inherited family condition. Keep the actual result and test date; ask about genetic counselling only when your personal or family history makes that relevant.

Sources: nccn-patient

Surgery, radiotherapy and medicine have different roles

Record what surgery removed and whether the present medicine is postoperative treatment to lower recurrence risk or treatment of cancer that remains or has spread. Radiotherapy, stereotactic treatment or another operation may have a separate role; their timing and side effects need their own plan.

Sources: nccn-patient

Stage and response are different

Stage describes the extent of melanoma at diagnosis or a later reassessment. A response assessment asks how known cancer has changed with treatment. In the adjuvant setting there may be no visible tumour to measure: surveillance looks for recurrence while safety checks look for medicine effects.

Sources: nccn-patient

A new skin change still needs review

Continue to report a new or changing mole, a non-healing lesion, a new lump or an unexplained persistent symptom. Do not assume every new lesion is melanoma, and do not use a photograph or a good response at one site to rule out a problem elsewhere.

Sources: nccn-patient

TREATMENT REFERENCE · 02

Your medicines and the overall plan

Nivolumab and ipilimumab help immune cells recognise and attack melanoma. The combination phase is followed by nivolumab alone when appropriate. The aim in this setting is to control advanced disease; benefit and toxicity are reviewed for the individual.

The schedule in this setting

The combination phase gives nivolumab followed by ipilimumab by IV infusion every three weeks for up to four doses. If appropriate, nivolumab alone then continues on the team-selected two-weekly or four-weekly schedule, by the prescribed IV or subcutaneous route. After the last combination dose, the first two-weekly maintenance dose starts three weeks later; the first four-weekly maintenance dose starts six weeks later in this melanoma schedule. Treatment continues until progression or unacceptable toxicity, subject to review.

Sources: nccp

How the treatment works

Nivolumab blocks PD-1, a signal that can reduce T-cell activity. This can help immune cells act against melanoma, but it can also allow inflammation in healthy organs. It is not conventional chemotherapy and side effects can occur even after doses have stopped. Ipilimumab blocks CTLA-4, another checkpoint involved in controlling T cells. Combining it with nivolumab increases immune activity and can increase immune-related toxicity. The four-dose combination phase has a different treatment plan from nivolumab maintenance.

Sources: nccp

The role of earlier or later treatment

Record surgery, radiotherapy and other anticancer medicines separately, including dates and the specialist responsible. This schedule does not automatically include or authorise another treatment. Ask whether any overlap changes tests, symptoms to report or the timing of appointments.

Sources: nccn-patient

Pauses and stopping

A safety pause is an active treatment decision. Record which medicine is paused, the reason, the tests or recovery needed and who may authorise restarting. Do not restart from an old prescription or reset the history when a new phase begins.

Sources: nccp

TREATMENT REFERENCE · 03

Taking treatment and preparing for visits

Keep the written medicine plan beside this guide; it records your own dose and dates.

Taking or receiving the medicine

At each combination visit, nivolumab is given before ipilimumab, as separate IV infusions. The source schedule gives each over 30 minutes; allow extra time for assessment, preparation and observation. Maintenance may be IV or subcutaneous nivolumab with its own dose and appointment interval. The route and interval are recorded in your plan; they are not interchangeable on your own.

Sources: nccp

Missed, vomited or held treatment

If an appointment is missed or treatment is held, contact the unit for a new plan. Do not count a missed visit as a dose received. A move to maintenance does not erase previous immune toxicity or a permanent-stop decision.

Sources: nccp

Other medicines and supplements

Show the pharmacist a current list of prescriptions, over-the-counter medicines, vitamins, herbal products and supplements. Include anticoagulants, heart medicines, steroids and recent antibiotics. Check before adding, stopping or changing anything; an apparently harmless product can alter treatment or confuse a symptom assessment.

Sources: nccp

Practical preparation

Arrange medicine supply, transport, interpreter or carer support before it is needed. Bring the alert card, symptom record and questions. Ask which appointments require someone to accompany you and how to obtain urgent advice outside normal hours.

Sources: nccn-patient

TREATMENT REFERENCE · 04

Tests: what is required and what each result means

Safety tests, tumour tests and response scans answer different questions.

Before treatment

Full blood count, kidney and liver tests, glucose and thyroid tests; BRAF tumour testing and review of autoimmune conditions, transplantation, prior immune reactions and current medicines.

Sources: nccp

During treatment

Full blood count, kidney/liver tests, glucose and thyroid tests each treatment cycle. Cortisol and other tests are added when clinically indicated. Every visit also reviews bowel, breathing, skin, endocrine, neurological and other symptoms; a normal blood test does not exclude immune toxicity.

Sources: nccp

Scans and tumour markers

Ask the team to write the next assessment and who will arrange it. Bloods used to monitor kidneys, liver, hormones or muscle are not automatically measures of cancer response. There is no universal tumour-marker blood schedule for these skin-cancer regimens; a scan or extra blood test is requested for a specific clinical reason.

Sources: nccp

After a pause or finish

Continue to report possible immune effects for at least five months after the last dose, as specified by NCCP, and for longer whenever your team advises. Keep any steroid taper, hormone-replacement plan and relevant follow-up tests. Response imaging follows your individual plan.

Sources: nccp

Getting the result

For each test, record its purpose, due date, result and action. Agree how results reach you and who checks a result from another hospital. If an appointment or result has not arrived, contact the named service rather than assuming no news means no action is needed.

Sources: nccn-patient

TREATMENT REFERENCE · 05

What to expect between visits

You can feel well and still need the planned checks. New symptoms deserve attention even on a day without an appointment.

Your usual pattern

Record the start and change in symptoms, temperature if advised, bowel pattern, drinking, urine and the effect on normal tasks. A simple few-line record is enough. Use it to describe a change rather than trying to grade or diagnose a toxicity yourself.

Sources: nccp

When to call

Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy.

Sources: nccp

Eating, drinking and activity

Tell the team early about nausea, taste change, a sore mouth, diarrhoea or falling weight. Ask for an individual fluid and food plan if heart, kidney or other conditions limit intake. Gentle activity and planned rest can help maintain function; adjust to symptoms and ask for support when fatigue limits daily life.

Sources: nccn-patient

A held dose and a new phase

If an appointment is missed or treatment is held, contact the unit for a new plan. Do not count a missed visit as a dose received. A move to maintenance does not erase previous immune toxicity or a permanent-stop decision.

Sources: nccp

TREATMENT REFERENCE · 06

Skin checks, sun protection and optional photographs

This complements clinical assessment and does not replace it.

Protect your skin

Use shade, protective clothing, a hat and broad-spectrum sunscreen of at least SPF 30, and avoid tanning beds. Reapply sunscreen according to its instructions. Protection matters even when it is cloudy; choose a practical approach that lets you remain active safely.

Sources: nccn-patient

Know the pattern of your skin

Ask how to examine your skin, scars and relevant lymph-node areas and how often your own team recommends doing so. A mirror or a trusted helper can make hard-to-see areas easier to check. Report a new or changing lesion, a non-healing sore or a new lump rather than waiting for a routine visit.

Sources: nccn-patient

Photographs are optional

If useful, keep a dated photograph with consistent lighting and a size reference, following your team's advice. Photographs are optional and do not diagnose a cancer or replace examination. Keep them privately; this printable passport does not require you to upload images or other personal health information.

Sources: nccn-patient

Wounds, nails and sensitive skin

Use the dressings and skin-care plan recommended for your lesion and treatment. Ask before applying new creams or remedies to an open or treated area. Worsening pain, discharge, redness, bleeding or loss of function needs advice; routine skin-care instructions are not a substitute for assessment.

Sources: nccn-patient

TREATMENT REFERENCE · 07

Pregnancy, fertility and protecting others

Discuss these precautions before treatment even if pregnancy is not currently planned.

Medicine-specific precautions

Discuss fertility and pregnancy precautions before starting. Immunotherapy may harm a pregnancy. Use the contraception plan and after-treatment duration specified for the medicines you receive; ask about breastfeeding restrictions. Tell the team promptly about possible pregnancy. Do not stop necessary hormone replacement or prescribed steroids without advice.

Sources: nccp

Planning ahead

Ask early about fertility preservation, sexual health and symptoms affecting intimacy. Tell the team about any chance of pregnancy, breastfeeding or a partner's pregnancy so that the correct medicine-specific advice can be given. Keep the precautions and their end dates in the written plan.

Sources: nccp

Safe handling and supply

Do not share anticancer medicines. Keep oral medicines in their original labelled packaging, away from children and anyone who could take them accidentally. Return unused medicines to the supplying pharmacy and follow its handling advice. A treatment hold does not remove pregnancy precautions.

Sources: nccp

TREATMENT REFERENCE · 08

Questions, decisions and practical support

Your priorities and the burden of treatment belong in the discussion.

Benefits and uncertainties

Ask what evidence applies to your diagnosis, stage, biomarker and treatment setting, what outcome the team hopes to improve, and what may happen without this treatment. A trial average cannot predict an individual result; a benefit measured in one setting should not be transferred to another.

Sources: nccn-patient

Alternatives and review points

Discuss other suitable treatments, clinical trials if relevant, local treatment options and symptom-focused care. Ask when the decision will be reviewed and what would prompt a change. Supportive and palliative care can help with symptoms and quality of life alongside anticancer treatment.

Sources: nccn-patient

Help with daily life

Tell the team about work, transport, caring responsibilities, cost concerns, body image, anxiety or sleep. Ask which nurse, social worker, dietitian, rehabilitation professional or support organisation can help. Use resources relevant to your country and service; a US guide may describe different access or funding.

Sources: nccn-patient

Shared understanding

Ask someone on the team to check that you can explain the medicine schedule, next tests and urgent-contact route in your own words. An interpreter, large-print copy or a trusted supporter can help; none requires you to use a smartphone.

Sources: nccn-patient

TREATMENT REFERENCE · 09

Follow-up and your continuing record

Keep this record when treatment is paused, changed or completed.

Your follow-up plan

Follow-up depends on stage, treatment and symptoms. Your team records the skin/lymph-node examination plan, whether body or brain imaging is needed, and which service owns each appointment. A general patient booklet does not set a universal scan interval for every patient.

Sources: nccn-patient

Continuing medicine precautions

Continue to report possible immune effects for at least five months after the last dose, as specified by NCCP, and for longer whenever your team advises. Keep any steroid taper, hormone-replacement plan and relevant follow-up tests. Response imaging follows your individual plan.

Sources: nccp

Treatment summary for other clinicians

Record the medicines and last dates, important reactions, any long-term hormone replacement or steroid taper, and the responsible oncology contact. Show this information in urgent care and to other specialists. A different treatment phase does not erase earlier toxicity.

Sources: nccp

The complete paper version

This handbook can be used without a phone: keep the schedule, symptom advice, medicine list, tests and contacts together. The QR code is an optional route to the matching regimen information. Ask the team to print updates when your plan changes and keep the latest dated copy.

Sources: nccn-patient

What the evidence means for your treatment

The evidence discussion must match the diagnosis, biomarker and treatment setting shown in this guide.

Ask which outcomes matter in your setting and what benefit is realistic for you. No survival percentage from another regimen, dose combination or disease setting is assigned here.

Sources: nccp · nccn-patient

PREPARE FOR THE CONVERSATION

Questions worth asking

What is the aim of my treatment?

Nivolumab and ipilimumab help immune cells recognise and attack melanoma. The combination phase is followed by nivolumab alone when appropriate. The aim in this setting is to control advanced disease; benefit and toxicity are reviewed for the individual.

Is there a fixed number of cycles?

The combination phase gives nivolumab followed by ipilimumab by IV infusion every three weeks for up to four doses. If appropriate, nivolumab alone then continues on the team-selected two-weekly or four-weekly schedule, by the prescribed IV or subcutaneous route. After the last combination dose, the first two-weekly maintenance dose starts three weeks later; the first four-weekly maintenance dose starts six weeks later in this melanoma schedule. Treatment continues until progression or unacceptable toxicity, subject to review.

What happens if treatment is held?

If an appointment is missed or treatment is held, contact the unit for a new plan. Do not count a missed visit as a dose received. A move to maintenance does not erase previous immune toxicity or a permanent-stop decision.

When are my next tests?

Full blood count, kidney/liver tests, glucose and thyroid tests each treatment cycle. Cortisol and other tests are added when clinically indicated. Every visit also reviews bowel, breathing, skin, endocrine, neurological and other symptoms; a normal blood test does not exclude immune toxicity. Your team adds the actual dates.

Can I use this without a smartphone?

Yes. The full handbook, three-page summary and wallet card are printable. The QR code is optional; keep actual contacts and written appointments on the paper copy.

> **Try explaining the plan back:** Can you explain your treatment setting, medicine schedule, next tests, the symptoms that need urgent advice and the number you would call?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00431 v11. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00431 v11: current regimen source
- NCCN Guidelines for Patients: Melanoma, 2026 - supplied education reference

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-06.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6734/431_Nivolumab_1mg_Ipilimumab_3mg.pdf

https://www.nccn.org/patients/guidelines/content/PDF/melanoma-patient.pdf